

Surgery for thymus cancer (thymectomy)

Surgery is the most common treatment for thymus cancer. The type of surgery you have depends on the size of the cancer and where it is in your body.

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Thymectomy

The main treatment for [thymus cancer](#) is surgery. The surgeon removes all of the thymus, or as much of it as possible. This operation is called a thymectomy.

If the cancer has not spread outside the thymus, this may be the only treatment you need. If the cancer has spread outside the thymus, the surgeon may need to remove nearby tissue around the lungs or heart.

If the cancer comes back in the same area, it may be possible to remove it using surgery.

Your surgeon will explain more about the operation you will have.

For the types of surgery mentioned below, you will usually have a general anaesthetic. We have more information about [preparing for surgery](#).

There are different ways in which a thymectomy can be performed. These include:

Video-assisted thoracoscopic surgery (VATS)

If the tumour is small, it can sometimes be removed using a type of keyhole surgery called video-assisted thoracoscopic surgery (VATS).

The specialist surgeon makes several small cuts at the side of the chest and passes a thin tube called a thoracoscope through one of the cuts. The tube has a tiny camera on the end which shows a picture of the area on a screen. The surgeon removes the thymus using small instruments that they pass through the cut.

VATS can also be used to remove the thymus by making a cut through the breastbone (sternum).

Open surgery

If the tumour is large or has spread outside [the thymus](#), you need open surgery. The surgeon makes a slightly bigger cut down the middle of the chest to remove the thymus and sometimes nearby tissue. This is called a sternotomy.

Robotic surgery

[Robotic surgery](#) is when keyhole surgery is helped by a machine. Instead of the surgeon holding the laparoscope and the surgical equipment, these are attached to robotic arms. The surgeon controls the robotic arms, which can move very precisely.

This is very specialised surgery and only available in some hospitals in the UK.

Recovery after surgery for thymus cancer

How long you are in hospital for depends on the operation you have. You are usually cared for on the ward but may be in a high dependency unit for a short while. This will depend on the type of surgery you have.

If you had open surgery, you may stay in hospital for up to 7 days. With keyhole surgery, you recover more quickly, so you may only need to stay in hospital for 1 or 2 days.

If you had open surgery, you usually have a drip (infusion) giving you fluids until you are able to drink normally. You may also have 1 or more tubes in the wound on your chest to drain fluid and air into a big bottle. This is called a chest drain. Your chest drain is usually removed 2 to 3 days after surgery.

After your operation, the nurses and physiotherapist will encourage you to start moving around as soon as possible. This will help prevent complications such as a blood clot or chest infection.

After surgery, it is normal to have some [pain](#) or discomfort. There are different ways it can be managed. Your nurse will assess you to make sure your pain is well controlled. If you are in pain, let your doctor or nurse know, so they can change your [painkillers](#).

The stitches, clips or staples are usually removed from your wound about 7 to 10 days after surgery. If you have dissolvable stitches, they disappear over a few weeks.

It takes time to recover from surgery and you may feel tired for several weeks. Before you go home, your doctor or nurse will give you advice about your recovery.

We have more general information about [what to expect after surgery](#).

About our information

References



Below is a sample of the sources used in our thymus cancer information. If you would like more information about the sources we use, please contact us at cancerinformationteam@macmillan.org.uk

ESMO Annals of Oncology. Thymic Epithelial Tumours: ESMO Clinical Practice Guidelines for diagnosis, treatment and follow-up: 2015. Available from <https://www.esmo.org/guidelines/esmo-clinical-practice-guideline-thymic-epithelial-tumours> (accessed July 2021)

Marx A, Chan JK, Coindre JM, et al. The 2015 World Health Organization Classification of Tumours of the Thymus: Continuity and Changes. Journal of Thoracic Oncology 2015; 10: 1383-95 (accessed July 2021)

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Reviewers



This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer. It has been approved by Senior Medical Editor, Dr David Gilligan, Consultant Clinical Oncologist.

Our cancer information has been awarded the [PIF TICK](#). Created by the Patient Information Forum, this quality mark shows we meet PIF's 10 criteria for trustworthy health information.

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

Date reviewed

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⚠ This content is currently being reviewed. New information will be coming soon.

Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

How we can help

Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help